



CHANDAN HOSPITAL

NABH HAND BOOK

“You are a part of our team”

You are a part of organization which is in process of establishing an outstanding reputation for itself in medical care and quality services.

To achieve this goal, we have decided to be in NABH program which will improve our Services, professional growth and patient care.

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PART A

GENERAL TOPICS APPLICABLE TO ALL EMPLOYEES OF THE HOSPITAL

Our Vision :

"We at CHANDAN are COMMITTED to achieve EXCELLENCE in **QUALITY** of services and STRIVE for **continuous improvement** for providing world-class healthcare services and complete **customer satisfaction**"

Our Mission :

To provide compassionate, accessible, affordable, high quality healthcare services to the community.

Core Values:

We are committed to ESIC

- E : Ethics
- S : Services
- I : Infrastructure
- C : Cost affordability

Scope of services i.e. LIST OF ALL SERVICES OFFERED

Anaesthesiology
Dermatology And Venereology
Emergency Medicine
General Medicine
General Surgery Including Laproscopic Surgery
Obstetrics And Gynaecology Including IVF
Ophthalmology
Orthopaedic Surgery With Joint Replacement Surgeries And Spine Surgery
Otorhinolaryngology
Paediatrics
Psychiatry
Respiratory Medicine
Cardiology Including Cath Lab
Medical Gastroenterology
Nephrology Including Dialysis
Neurology
Neurosurgery
Surgical Oncology
Plastic And Reconstructive Surgery
Surgical Gastroenterology
Urology Including Lithotripsy
CTVS
Endocrinology
Dental-Oral And Maxillofacial Surgery
Critical Care [ICU,CCU,NICU,PICU]
Physiotherapy
Dietetics

24 HOURS SERVICES

Emergency

Ambulance

Pharmacy

Blood Bank

Dialysis

Diagnostic Services-

- Laboratory - Biochemistry, Clinical Pathology, Hematology, Immunology, Serology
- Radiology - X Ray, CT, MRI, USG, Doppler, BMD, OPG, Mammography
- Other- PFT, EEG, Audiometry, ECG, TMT, ECHO

1. WHAT ARE PATIENT RIGHTS AND RESPONSIBILITIES ?

Patient & Family Rights :

1. Respect for any special preferences, spiritual and cultural needs.
2. Respect for personal dignity and privacy during examination, procedures and treatment.
3. Protection from neglect or abuse.
4. Treating patient information as confidential.
5. Refusal of treatment
6. Right to seek an additional opinion regarding clinical care.
7. Informed consent before transfusion of blood and blood components, anesthesia, surgery, any other invasive / high risk procedures / treatment.
8. Right to complain and information on how to voice a complaint.
9. Information on the expected cost of the treatment.
10. Access to his / her clinical records.
11. Information on Care plan, progress and information on their health care needs.

Patient Responsibilities :-

1. To provide accurate information about disease and symptoms.
2. To follow the healthcare professional instructions.
3. To follow the hospital rules and regulations.
4. To pay hospital dues on time.
5. To maintain hospital decorum & help hospital administration maintains the same.
6. To respect hospital staff & other patient

2. WHAT ARE EMPLOYEE RIGHTS & RESPONSIBILITIES ?

EMPLOYEE RIGHTS

- To be aware of the hospital policies.
- To be treated considerately and respectfully, without any discrimination on the basis of caste, religion, gender or socio-economic background
- To be aware of the terms and conditions of his/her employment before joining the organization and awareness about service rule.

- If any one believes that he/she has been the victim of any kind of harassment or knows of another employee who has the right to, report it immediately to the HR Department.
- To seek clarity on the targets to be achieved and the roles/responsibilities associated with the task to be performed

EMPLOYEE RESPONSIBILITIES

- To follow and respect the hospital rules and regulation.
- Employees are expected to work on shift or their duty hours to support the hospital's 24*7 operations.
- Employees shall be responsible for the equipment allocated to them and maintain it in accordance with the standard operating procedures.
- Employees are expected to maintain proper discipline, professional ethics and integrity in their work.

3. WHAT IS PATIENT'S GRIEVANCE POLICY ?

A formal, written or verbal grievance is filed by a patient/kin when there is dissatisfaction with hospital services & is reported to Chief Admin OPD/IPD VIA feedback forms and floor coordinators via feedback also we have installed complaint box where patient can drop there complaint .they can also report to telcaller dept.

4. WHAT IS EMPLOYEES GRIEVANCE POLICY?

All complaints arising out of employment including those relating to unfair treatment and wrongful application of these service rules shall be submitted to the top management.

The employees shall exhaust the above procedure before resorting to any legal remedy.

- 1st Step: Complaint to your incharge/HOD
- 2nd Step: Complaint to MS/HR
- 3rd Step : Complaint to Director – Administration
- 4th Step : Approach to the Appellant Authority (Managing Director) His decision will be final.

5. WHAT ARE EMERGENCY CODES ?

It is the codes given to different emergency situations arising in the hospital to avoid panic in the hospital and to make employees understand the situation .

RED	FIRE
PINK	CHILD ABDUCTION
BLUE	CARDIAC ARREST
VIOLET	VIOLENCE
YELLOW	DISASTER etc

[CALL 999 FOR ALL]

6. HOSPITAL WIDE POLICIES :-

- No Smoking policy
- Outside food is not allowed for patients
- Outside pharmacy is not allowed
- Children below 12 years are not allowed as visitors in ICU

7. HOSPITAL COMMITTEES

1: Quality Improvement Committee

This committee looks after the Quality Management Program that is systematic organization wide & consistent with the mission, vision & the values of the hospital.. This committee will also look after the continuous quality improvement in the hospital.

2: Safety Committee :

Safety committee is responsible for overall maintaining the safety of the patient, visitors, attendant, and staff . To identify the potential safety and security risks within the Hospital.

3: Infection Control Committee:

Infection control committee is responsible for maintaining the overall infection control in the hospital. Committee will also ensure regular training program on the infection control to the various staff.

4: Medical Record Committee-

Medical record committee is responsible to oversee and maintaining the quality of medical records that's include inpatient files OPD files and other forms/formats and registers etc used in the hospital.

5: Pharmacy And Therapeutic Committee

This committee is responsible to handle all the issues related to the drugs like adverse drug reactions, adverse drug events, maintaining the quality of crash cart, developing the emergency medication list, high risk drugs list and look alike and sound alike medication list etc.

6: Anti Sexual Harassment Committee- NOW NEW NAME IS INTERNAL COMPLAINT COMMITTEE

Anti-sexual harassment committee is responsible for handling the female grievances (sexually harassment) any kind by any staff. In case of any harassment staff can bypass grievance policy and directly inform any of the members of this committee given below-

7: CPR Committee

To ensure an effective hospital-wide Cardio Pulmonary Resuscitation (CPR) program, To ensure that policies and procedures related to CPR are consistently followed throughout the organization.

8: Biomedical Waste Management Committee

To ensure effective management & handling of biomedical waste starting from collection to final disposal generated at different sites in the hospital. To ensure proper monitoring of infected wastes at source .

8. FIRE SAFETY

In case of fire

RACE

R : Rescue

A Alarm

C : Confine

E: Extinguish/Evacuate

PASS

P : Pull the pins between the handle

A : Aim the nozzle at the base of the fire

S : Squeeze the handle

S : Sweep from side to side

Call for help, call code red by dialing 999

While calling tell your name also and ask operator names[quickly].then say-

Code red-floor-dept/ location

where fire is incident. Repeat 3 times

Fire Extinguisher- ABC is Multipurpose



FIRE MOCK DRILLS HAVE TO BE CONDUCTED TWICE IN A YEAR AS PER LEGAL NORMS

9. WHO IS VULNERABLE PATIENT ?

“Those patient who are prone to injury and disease by virtue of their age, Sex, Physical, mental and immunological status.”

e.g.:-

- Children < 12 yrs.
- People > 65 yrs of age.
- Patient on immunosuppressive or chemotherapeutic agents
- Pregnant patients
- Physically and mentally challenged
- Rape/suicide victims
- Patient in ICU
- Patient who cannot perform activity of daily living. (ADL)

For Vulnerable patient, precautions taken are-

- Side rails on bed+ appropriate bed of safe height+ mattress/overlay.
- Must be attended by family attendant/nurse.
- Provided scheduled toileting + scheduled turning and positioning.
- Urine and / or feces cleaned promptly.
- Skin care and hygiene.
- Provide timely nutrition /feeds
- Attended for physical interventions i.e. bathing, toilet by appropriate gender
- Restraints if required comfort assessment and emotional care by psychologist.
- Should be identified as vulnerable (orange band)

ANY ABUSE MUST BE REPORTED TO CONCERNED DEPARTMENT

10. SENTINEL EVENTS

A relatively infrequent, unexpected incident, related to system or process deficiencies, which leads to death or **major and enduring loss of function** for a recipient of healthcare services. **Major and enduring loss of function** refers to sensory, motor, physiological, or psychological impairment not present at the time services were sought or begun. The impairment lasts for a minimum period of two weeks and is not related to an underlying condition.

Example :-

- Surgical events-ex wrong side surgery
- Device or product-ex blood transfusion reaction
- Patient protection-ex hospital acquired infections,patient suicide/injury
- Environment-ex burn,fall,electric shock
- Care management-ex medication error
- Criminal-ex rape

all such events must be reported by incident report forms available with department in charges to safety officer after taking appropriate actions

11. WHAT DO YOU MEAN BY INDUCTION PROGRAM ?

It is an introduction given to all new joining employees including overview of the hospital and its facilities within 15 days of the joining from hospital side.HR dept conducts induction trainings.

12. STAFF HEALTH PROGRAMS IN THE HOSPITAL

- Pre-employment health checkup:- it is done at the time of recruitment of employees .
- Annual health checkup : It is done once in a year for all who are working in patient related areas only eg nurse, doctor, technician, House keeping staff and ward attendant etc
- For health policy refer service rule book/HR dept
- Pre exposure prophylaxis is given to staff i.e. Hepatitis vaccine & Injection T.T.
Kitchen staff : Typhoid and Hepatitis A vaccines.

13. WHAT IS THE ATTIRE CODE ?

It is the attire protocol which is to be followed in to prevent infection

- **ICUs** Gowns, face mask, head cover, shoe cover or slipper.
- **OT** scrub suit, face mask, head cover, OT slippers.
- **Other:** patient handling with mask & gloves.
- **ICU-** patient dress- Pyjama& top.

14. WHAT ARE OCCUPATIONAL HAZARD ?

- Biological hazards (nsi-needle stick injury,blood, body fluids exposure)
- Chemical hazards (formalin, cidex, mercury)
- Physical hazards (fall, back ache etc.)
- Safety hazards (electric hazard, broken equipment)
- Psychological hazards (work related stress)

15. OCCUPATIONAL HAZARDS OF HOSPITAL STAFF :

- Doctors & Nursing Staff : Hospital Acquired Infection (HAI), Needle Stick Injury (NSI)
- Lab/sample collection room : HAI, NSI, Blood/Chemical Spill
- Radiology : Radiation
- Maintenance : Electric Shock, HAI, Ear problems
- House Keeping : HAI, NSI
- Account/billing/Computer Work : backache, Eye strains
- Kitchen Staff : Fire

16. WHAT ARE QUALITY INDICATORS [KPIs-key performance indicators]?

Quality Indicators are a set of functional parameters which have been selected across different area of the hospital. Data for these is collected by the respective departments every month and analyzed by the Quality Committee, which looks into gross deviations in any of this data.

17. WHAT DOES NABH STANDs FOR ?

NABH: National Accreditation Board for Hospitals and Health Care Providers.

18. STRUCTURE OF NABH STANDARDS [full accreditation] ?

HCO Standards : (HCO-healthcare organization above 50 bedded hospitals)

- . 10 chapters
- . 106 Standards
- . 683 Objective elements

20. WHAT ARE THE 10 CHAPTERS OF NABH ?

Patient centered standards:-

- Chapter-1 **(AAC)** Access, Assessment and continuity of care
- Chapter-2 **(COP)** Care of Patient
- Chapter-3 **(MOM)** Management of Medication
- Chapter-4 **(PRE)** Patient Right and Education
- Chapter-5 **(HIC)** Hospital Infection Control

Organization centered Standards:

- Chapter-6 **(CQI)** Continuous Quality Improvement
- Chapter-7 **(ROM)** Responsibility of Management
- Chapter-8 **(FMS)** Facility Management and Safety
- Chapter-9 **(HRM)** Human Resource Management
- Chapter-10 **(IMS)** Information Management System

APPLICABLE TO ALL EMPLOYEES WORKING IN PATIENT CARE AREAS



PART B

PATIENT SAFETY GOALS

GOAL 1 : IDENTIFY PATIENTS CORRECTLY

Use at least two (2) ways (**UHID & Patient's Name**) to identify a patient before:

- Administering Medications
- Administering Blood and Blood products
- Collecting Blood Samples
- Label containers used for Blood & other Samples/Specimens for Clinical Testing in the presence of the patient
- Providing Treatment or Procedures
- Serving Special/Therapeutic diet to the patient

Room Number & Physical Location should not be used as an identifier.

GOAL 2 : IMPROVE EFFECTIVE COMMUNICATION

- Verbal Orders/Telephonic orders;follow Verbal Order Policy-
Listen Document Read back Confirm Counter Sign
- Reporting of Critical Test Results to the treating consultant.
- Mention the time of availability & time of informing the critical result.
- While providing Patient / Family Education explain them in the language and manner they understand.
- Hand Offs(between one health care provider to the health care provider)to be done at each patient's bed side with all relevant details.

GOAL 3 : IMPROVE THE SAFETY OF USING MEDICATIONS

Remove Concentrated Electrolytes from patient care units, including, but not limited to the following:

- Potassium Chloride,Potassium Phosphate,Sodium Chloride>0.9%,Magnesium Sulphate>=5%conc.
- List of Look Alike Sound Alike (LASA) Drugs & High Risk Drugs to be kept at all nursing stations / clinical settings
- **Label all medicines**/containers/bottle solutions in Syringes, Cups and Basins & on & off the sterile field in the perioperative area.

- Verification of all medicines & solutions to be done both verbally & visually **by two qualified individuals.**
- Use appropriate protocols for the initiation & maintenance of the anticoagulant therapy.
- Written procedures & protocols to be followed for storage, prescription, preparation, administration & monitoring for High risk medications.

GOAL 4 : ENSURE CORRECT-SIDE / SITE, CORRECT PROCEDURE, CORRECT PATIENT SURGERY

- Ensure PAC checkup/Immediate Pre-Op Re-evaluation for all surgical procedures
- Ensure Pre-Operative verification by performing “**Time out**” Procedure– verify correct site, correct procedure and patient
- Ensure site/side is properly marked by the operative person and Pre-Operative checklist is duly filled.
- Ensure Images and Studies are available, properly labeled and displayed at the time of initiating the procedure.
- Ensure verification of any requirement of Special Equipment and Implants
- Ensure uniform implementation of “Surgical Safety Checklist” implementation for all surgeries, in all care settings.

GOAL 5: REDUCE THE RISK OF HAIs

- Use WHO Clean Hands: Save Lives Guidelines/CDC Guidelines to prevent infections
- Track hospital acquired infection rates and implement proven guidelines to prevent **CLABSI, CAUTI, VAP & SSI**
- Use appropriate Care Bundles, as per CDC guidelines
- Implement Engineering Controls Infection Prevention
- Ensure implementation of CSSD Protocols & Bio-Medical Waste Management
- Adhere to Universal Precautions, Follow barrier Nursing protocol for all infected cases and use appropriate PPEs in all care settings

GOAL 6 : REDUCE THE RISK OF PATIENT HARM FROM FALLS

- Assess each (OPD & IPD pt.) for **fall risk assessment** at the time of first OPD visit & at the time of admission.
- Periodically reassess each patient's risk for falling
- Recheck potential risk associated with the **patient's medication regime**.
- **Educate Pt./Pt's attendant for fall prevention** protocols and how to call for help, specially for all Vulnerable patients
- Conduct **safety rounds** to decrease or eliminate any identified risks, implement Hazard identification & risk analysis checklist.
- Improve the safety of clinical alarm system

CRASH CART

A movable collection of emergency equipment and supplies meant to be readily available for resuscitative effort .

It includes medication as well as the equipment for defibrillation, intubation, intravenous medication, and passage of central lines.

CARDIO PULMONARY RESUSCITATION

- With the new Compressions – Airway – Breathing method a victim receives compressions faster, providing quicker critical blood flow to the vital organs
- The sequence of steps for conducting CPR using the C-A-B method
- **“C” for Compressions**
 - Rate of compressions: You should give **at least 100 to 120 compressions per minute** in cycles (or sets) of 30 compressions and 2 breaths (30:2).
 - Depth of compressions: detailed in following chapters for adult, child and infant.

Chest recoil is vital after each compression.

4 TYPES OF DRUG LISTS?

- **Emergency drugs**
- **High risk drugs**
- **Look alike drugs**
- **Sound alike drugs**

SAFE INJECTION PRACTICES

- Step 1:** Clean work space.
- Step 2:** Hand hygiene.
- Step 3:** Sterile and new syringe and needle, with re-use prevention and/or injury protection feature whenever possible.
- Step 4:** Sterile vial of medication and diluent.
- Step 5:** Skin disinfection.
- Step 6:** Appropriate collection of sharps.
- Step 7:** Appropriate waste management.

By following these simple steps, you take key actions to ensure the risks of unsafe injections are avoided.

THE TEN "R'S" OF SAFE MEDICATION ADMINISTRATION



Two nurses administer the high risk medicine after cross checking especially Narcotic & psychotropic drugs. Never leave medicines unattended in the open. Lock them in beside cabinets, label all open, in use vials.

NAME THE PATIENT IDENTIFIER ?

1. UHID
2. Patient name

WHAT IS MSDS ?

- ✓ Material safety data sheet (It contains a list of all hazardous material)
- ✓ Hazardous Material : Substances dangerous to human and other living organisms.
- ✓ Example : Hypochlorite solution, Cidex, Diesel, radioactive material & chemical material etc
- ✓ Its tell how to store them and what to do when exposed to such hazardous material
- ✓ All dept have this MSDS list [if applicable to that dept] either displayed or in dept. directory file.
- ✓ One set is in emergency also.
- ✓ If exposure occurs,one must immediately do primary management like washing in running water and report to emergency.

NAME TWO RADIATION SAFETY DEVICES ?

- TLD badges
- Lead aprons

WHAT IS RADIATION SAFETY PRECAUTION

1. Use TLD Badges,
2. Use of Radiation Safety Devices ; Gonad shield, Thyroid Shield, Abdominal Shield and lead apron etc
3. Universal Precautions need to be followed.
4. Display of radiation signage
5. Screening of pregnant ladies
6. Comply with legal AERB Licenses

WHAT IS LAB SAFETY PRECAUTION?

1. Following Universal Precautions
2. Personal Protective Equipment
3. Vaccination or Pre exposure prophylaxis
4. BMW Management with proper segregation.
5. Fire safety
6. Spill management
7. Use and knowledge of MSDS.
8. Eye washer

WHAT IS CONSENT ? WHEN IT SHOULD IT BE TAKEN ?

A willingness of a patient to undergo examination/procedure/treatment by a health care provider. It may be implied/expressed and may be written or verbal.

Informed consent is a type of consent in which the health care provider has a duty to inform his/her patient about the procedures, its potential risks and benefits so as to enable the patient to take a informed decision of his/her health care.

WHO OBTAINS INFORMED CONSENT ?

The Doctors.

LIST OF SITUATIONS WHERE INFORMED CONSENT IS TAKEN ?

For anaesthesia,
All kind of procedures/surgeries
All kind of investigations involving risk to the life of patient
Blood transfusion
Blood Donation
HIV testing
MTP
Antenatal USG
Organ transplantation
Sterilisation
Family planning etc

CHECK-LIST OF KEY ASPECTS OF PRESCRIPTION:-

- Date, name of the patient at top and signature & time at bottom.
- Clear legible prescription in capital letters.
- Dose, route & duration of administration.

Note: Also check for allergies and note down

TURN AROUND TIME FOR INITIAL ASSESSMENT BY DIFFERENT CARE GIVERS

By Nurse :- Within 3 minutes after patient arrival in emergency

Within 30 minutes after patient arrival in wards

By Doctor :-

Within 7 minutes after patient arrival in emergency

Within 1 hr after patient arrival in wards

By Dietician :-

Within 24 hrs of patient arrival in emergency & wards

By Physiotherapist :-

Within 24 hrs of patient arrival in emergency & wards (where applicable)

TURN AROUND TIME FOR RE-ASSESSMENT BY DIFFERENT CARE GIVERS :-

It has to be done every 24 hrs /depending on patient need & condition

MLC POLICY :-

Medico legal cases (MLC) are an integral part of medical practice that is frequently encountered by Medical Officers (MO)

Proper handling and accurate documentation of these cases is of prime importance to avoid legal complications and to ensure that the Next of Kin (NOK) receive the entitled benefits.

A MLC is defined as “any case of injury or ailment where, the attending doctor after history taking and clinical examination, considers that investigations by law enforcement agencies are warranted to ascertain circumstances and fix responsibility regarding the said injury or ailment according to the law”

The decision to label a case as MLC should be based on sound professional judgement, after a detailed history taking and thorough clinical examination.

Examples of MLCs. The following are some of the examples of MLCs and medical officers should use their professional judgement to decide any other cases not enumerated in the list: (a) Assault and battery, including domestic violence and child abuse (b) Accidents like Road Traffic Accidents (RTA), industrial accidents etc. (c) Cases of trauma with suspicion of foul play (d) Electrical injuries (e) Poisoning, Alcohol Intoxication 3 (f) Undiagnosed coma (g) Chemical injuries (h) Burns and Scalds (j) Sexual Offences (k) Criminal abortions (l) Attempted suicide (m) Cases of asphyxia as a result of hanging, strangulation, drowning, suffocation etc. (n) Custodial deaths (o) Death in the operation theatre (p) Unnatural deaths (q) Death due to Snake Bite or Animal Bite (r) Fire Arm injuries (s) Drug overdose (t) Drug abuse (u) Dead brought to the Accident and Emergency Dept / MI Room (Found dead) and deaths occurring within 24 hours of hospitalization without establishment of a diagnosis

Medico legal documents should be considered as confidential records and should be stored under safe custody to avoid tampering. Medical records must be thorough, complete and should document each and every significant event in the course of care of the patient. All the documents including case sheets, X-rays and investigation reports will be preserved meticulously in the medical record section indefinitely and handed over to the concerned authorities (Police Investigating Officer / Court / Court of Inquiry) as and when required.

WHO ALL ARE AUTHORIZED TO WRITE DRUG ORDERS IN PATIENT FILE ?

Minimum MBBS & BDS doctors.

WHO ALL CAN DO ENTRY IN PATIENT FILE ?

Doctor, Nurse, Dietician and Physiotherapist.

COLOUR CODING IN TRIAGE:-

RED : Most Urgent

Yellow : Urgent

Green : Non Urgent

Black : Dead

WHAT DO YOU UNDERSTAND BY NOTIFIABLE DISEASES?

Certain specified diseases, which are required by law to be notified to the public health authorities. Under the international health regulation (WHO's International Health Regulations 2005) the following diseases are notifiable to WHO:

- (a) Smallpox
- (b) Poliomyelitis due to wild-type poliovirus
- (c) Human influenza caused by a new subtype
- (d) Severe acute respiratory syndrome (SARS).

In India, the following is a indicative list of diseases which are also notifiable, but may vary from state to state:

- (a) Polio
- (b) Influenza
- (c) Malaria
- (d) Rabies
- (e) HIV/AIDS
- (f) Louse-borne typhus
- (g) Tuberculosis
- (h) Leprosy
- (i) Leptospirosis
- (j) Viral hepatitis
- (k) Dengue fever

The various diseases notifiable under the factories act lead poisoning, byssinosis, anthrax, asbestosis and silicosis.

WHAT IS AN "ADVERSE EVENT" AND "ADVERSE DRUG REACTION" AND WHEN & WHOM TO REPORT SUCH EVENT ?

- **Adverse Drug Reaction** : A response to a drug which is noxious and unintended and which occurs at doses normally used in man for prophylaxis, diagnosis or therapy of disease or for the modification of physiologic function.

Therefore ADR = adverse event with a causal link to a drug.

Adverse Event: The FDA recognizes the term adverse drug event to be synonym for adverse event. In the patient safety literature the terms adverse drug event and adverse event usually denote casual association between the drug and the event, but there is a wide spectrum of definition for these terms including harm caused by a-

- Drug
- Harm caused by drug use, and
- A medication error with or without harm.

Institute of Medicine: "An injury from medical intervention related to a drug" which has been simplified to **"An injury resulting from the use of drug"**.

Adverse drug event extend beyond adverse drug reactions to include harm from overdoses and under doses usually related to medication errors.

Minorities of adverse drug events are medication errors,
And medication error rarely result in adverse drug event.

Reporting

It should be reported to the medical director/chairperson of pharmacy & therapeutic committee [MR ANANT SINGH] within 24 hrs of occurrence of such event

WHAT IS DRUG FORMULARY ?

An approved list of drugs. Drugs contained on the formulary are generally those that are determined to be cost effective and medically effective. The list is compiled by professionals and physicians in the field and is updated at regular intervals. Changes may be made depending on availability or market

RULES FOR ATTENDING FEMALE PATIENTS I.E PATIENT PRIVACY ;-

A doctor will always examine female patient in the presence of female nurse/attendant only .

RULE REGARDING PATIENT CONFIDENTIALITY :-

All the staff of the hospital is bound to maintain patient confidentiality, under no circumstances the patient condition will not be discussed or reviewed in front of the other visitor or relative without the prior permission of the patient

UNIFORM CARE POLICY :

In CHANDAN hospital all the patient will be treated and cared uniformly without any bias on the basis of caste, creed, religion, region, financial status etc. Uniform care will be given to all

NARCOTIC DRUG POLICY ?

Narcotic drugs :-

The term narcotic originally referred medically to any psychoactive compound with sleep-inducing properties . These drugs have habit forming property so there use is a crucial concern.

Examples : Fentanyl, Morphine and heroin .etc

These drugs are only given after doctor prescription and cant be stored if the hospital is not having license .

After administration of such drugs the empty vials or ampoules have to be stored and given to pharmacy who

will submit to concerned dept

POLICY FOR IMPLANTS AND PROSTHESIS ?

Three sticker containing batch & serial number have to be recoded in the patient medical record, the master logbook and the discharge summary .

WHAT DO YOU MEAN BY INCIDENT REPORTING ?

Reporting of any event occurred in the organization eg near miss, adverse event, sentinel events etc

AFTER TAKING IMMEDIATE APPROPRIATE ACTION THESE EVENTS MUST BE REPORTED TO SAFETY OFFICER.

POLICY FOR RETENTION OF MEDICAL RECORDS ?

IPD records	10 yrs
MLC	Lifelong
Expired	Lifelong

POLICY FOR ISSUE OF MEDICAL RECORDS TO PATIENTS/KINS?

A request is to be made on MRD request format and the documents requested will be issued within 72 hrs of the request made

WHAT DO YOU MEAN BY RESTRAINTS ?

Devices used to ensure safety by restricting and controlling a person's movement. Many facilities are —restraint free|| or use alternative methods to help modify behaviour. Restraint may be physical (by cotton bandage) or chemical (by use of sedatives).MONITORING NEEDED.

WHAT IS "END OF LIFE CARE" ?

When the doctors and patient representatives decide to withhold or withdraw the life support measures, it is documented in the patient's file with consent of the next kin of the patient.

Nurses carry out the orders and also ensure that the patient and family are taken care with utmost respect and compassion/palliative care.

WHAT IS NEAR EXPIRY DRUG POLICY ?

All the medicines which are due to expire in next three months are submitted from different areas of the hospital to pharmacy.[expire drugs are to be submitted to warehouse]

SELF ADMINISTRATION OF DRUG POLICY ?

Self administration of drugs by patient itself is not allowed it is only allowed under the supervision of the nurse. Medicines brought from home are not allowed.

VERBAL ORDER READ BACK POLICY ?

We have no verbal orders for nurses. We have RMOs

WHAT SHOULD BE TEMPERATURE OF REFRIGERATOR (MEDICINE) ?

- 2-8 degree Celsius
- (Exception is Polio Vaccine which is stored in ice box below 0 degree Celsius).

DEFINE :-

PROTOCOL

A plan or a set of steps to be followed in a study, an investigation or an intervention.

PROCESS

A set of interrelated or interacting activities which transforms inputs into outputs (Para 3.4.1 of ISO 9000: 2000).

POLICIES

They are the guidelines for decision-making, e.g. admission, discharge policies, antibiotic policy, etc.

PRIVILEGING

It is the process for authorizing all medical professionals to admit and treat patients and provide other clinical services commensurate with their qualifications and skills.

PROCEDURE

A specified way to carry out an activity or a process (Para 3.4.5 of ISO 9000: 2000).

A series of activities for carrying out work which when observed by all help to ensure the maximum use of resources and efforts to achieve the desired output.

PERFORMANCE APPRAISAL

It is the process of evaluating the performance of employees during a defined period of time with the aim of ascertaining their suitability for the job, potential for growth as well as determining training needs.

NEAR-MISS

A near-miss is an unplanned event that did not result in injury, illness, or damage--but had the potential to do so. Errors that did not result in patient harm, but could have, can be categorised as near-misses.

MEDICATION ERROR

A medication error is any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is in the control of the health care professional, patient, or consumer. Such events may be related to professional practice, health care products, procedures, and systems, including prescribing; order communication; product labeling, packing and nomenclature; compounding; dispensing; distribution; administration; education; monitoring; and use.

INCIDENT REPORTING

It is defined as written or verbal reporting of any event in the process of patient care, that is inconsistent with the deserved patient outcome or routine operations of the healthcare facility.

HIGH RISK /HIGH ALERT MEDICATIONS

High-risk / high-alert medications can be defined as those drugs that have a heightened risk for adverse events

or have heightened risk of catastrophic harm whenever there is an error. These drugs include generally have low therapeutic index .

CREDENTIALING

The process of obtaining, verifying and assessing the qualification of a healthcare provider.

PART C

APPLICABLE TO ALL DEALING WITH INFECTION CONTROL PRACTICES

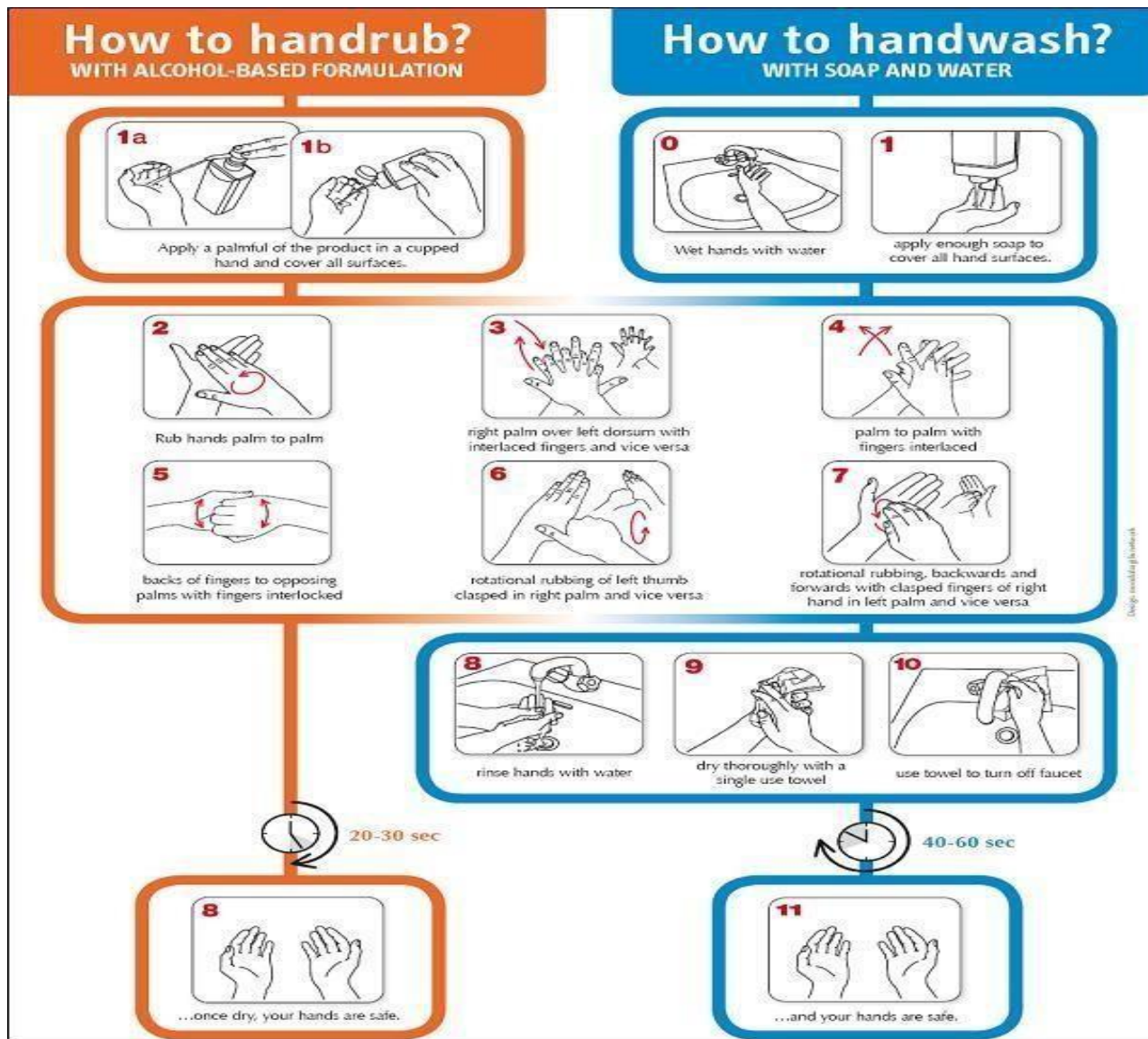
INFECTION CONTROL IN HOSPITAL

- It comprises infection control program

- Infection control program is updated atleast once in a year
- It includes infection control committee
- Infection control team includes **Infection control officer** and the **infection control nurse**
- High risk areas in the hospital comprises :-

ICU, Dialysis, Blood bank, HDU, OT, labor room post operative ward, CSSD etc

HAND HYGIENE TECHNIQUE:



2. HOW LONG IT TAKES TO DISINFECT YOUR HANDS ?

- Alcohol base hand rubs- 20 seconds.

- Anti microbial soap and water- 1minute

2. Five (5)MOMENTS FOR HAND HYGIENE

This hand hygiene approach defines the key moments when health-care workers should perform hand hygiene. This approach recommends health-care workers to clean their hands:

1. Before touching a patient
2. After touching a patient
3. Before clean / aseptic procedures
4. After body fluid exposure / risk
5. After touching patient's surroundings



3. WHAT IS NEEDLE STICK INJURY & WHAT SHOULD BE DONE IN CASE OF NEEDLE STICK INJURY ?

Needle stick injuries are wounds caused by needle/sharps that accidentally puncture the skin. Steps to be followed in case of needle stick injury are:-

- Do not panic.
- Do not put injured part in mouth as a reflex.
- Wash the area with soap and running water .
- Report to the infection control nurse / EMO in casualty.
- Record details of exposure and source,
- Follow the needle stick injury policy in management of sharp injuries.
- Contact the infection control nurse/officer for any information on the same.

4.)RE AND POST EXPOSURE PROPHYLAXIS

- ▶ Pre Exposure Prophylaxis : Vaccination (Hepatitis B, T.T. etc)
- ▶ Post Exposure Prophylaxis : A.R.T (Antiretroviral Therapy)

5. PERSONNEL PROTECTIVE EQUIPMENT (PPE)

Personal Protective Equipments (PPE) refers to self protection designed to protect the wearer's body from job-related occupational injuries occurring by electrical hazards, heat, chemicals, infections, etc.

These are the equipment's which are designed to protect healthcare workers from Blood and Body Fluid exposure.

PPE:

FOR CLINICAL STAFF-

- Cap (Head protection)
- Goggle (Eye protection)
- Mask (Respiratory Protection)
- Apron (Body protection)
- Gloves (Hand protection)
- Shoe covers

FOR OTHER STAFF-

- Filter mask/gas masks (respiratory protection)
- Helmets/head guard/cap (head protection)
- Goggles/eye protector (eye protection)
- Ear plugs/muffs (ear protection)
- Gum boots

6. BIO MEDICAL WASTE (BMW) MANAGEMENT

What is Bio-Medical Waste & how should it be disposed ?

Bio-Medical waste is the waste that is generated during the diagnosis, treatment or immunization of human beings or animals, or in research activities pertaining thereto, or in the production or testing of biological matter. It must be disposed within 48 hours of collection and must be segregated as per hospital policy.

Four Categories :

- 1. Yellow**
- 2. Red**
- 3. White**
- 4. Blue**

7. WHAT ARE DO'S AND DON'TS WHILE HANDLING SHARPS ?

Do's-

- Pass syringe & needles in a tray.
- Remove cap of needle near the site of use.
- Pick up open needles from tray with forceps.
- Use puncture proof containers near site of usage to collect & dispose needle & the sharp/broken ampoules.

Don'ts-

- Never pass syringe & needles directly to next person.
- Do not bend/break used needle with hands.
- Never test fineness of needle/ pick up open needle by hand
- Never dispose it off by breaking with hammer/stone.

8. SPILL MANAGEMENT

STEPS TO SPILL MANAGEMENT



Wear suitable PPE

Outline the Spill material by Marker pen



Cover spill fluids with 1% of freshly prepared sodium hypochlorite for 10 minutes

Pick-up absorbed material & place in disposal bag



Mop dry & a second decontamination may be done if required

Wash the area with detergent and water



Prepare spill report & Report immediately to CMO & ICN



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9. WHAT DO YOU MEAN BY NOSOCOMIAL/HOSPITAL ACQUIRED INFECTIONS (HAI,S) ?

Hospital associated infections (HAI) or nosocomial are infections that are acquired by the patient while he/she is in the hospital either from other patients, the hospital environment or members of the hospital staff. These infections usually manifest after 48 hours of admission. Some may manifest even after discharge from the hospital. EX-CAUTI,CLABSI,VAP etc

Types

- Urinary tract infections.
- Respiratory infections.
- Surgical site infections.
- Device related infections.
- Bloodstream infections.

IMPORTANCE

- Hospital acquired infections are those which considered as a major causes of enhanced mortality, morbidity, and emotional stress in hospitalized patients.
- The average length of stay of patients and the cost of health care also increases significantly, thereby leading to loss of effectiveness and productivity of the hospital services.
- Hospital acquired infections also result in significant economic loss and additional burden on health care institutions.
- With the recent changes in the customer protection act, health care facility runs the risk of being the target of litigations by the patients
- It also earns a bad reputation for the hospital

SOURCE OF INFECTION

- The patients own resident flora, the mouth, gastrointestinal tract, vagina or the skin.
- The resident microbial flora of health care workers and from other patients on the ward.
- Transient bacteria carried on the hands of the health care workers from one patient to another.
- Infusions.
- Contaminated instruments, dressings, needles, etc. used for invasive procedures.

10. HIGH RISK AREAS

- Operation Theatre

- Post Operative Room
- CSSD
- ICU
- ICCU
- NICU
- PICU
- Dialysis
- Labor Room
- Blood Bank
- Laboratory

11. PREVENTION OF HOSPITAL ACQUIRED INFECTION

- Reducing person to person transmission.
- Preventing transmission from the environment

12. STANDARD PRECAUTIONS

Standard precautions are meant to reduce the risk of transmission of blood borne and other pathogens from both recognized and unrecognized sources :-

It also includes the use of Personal protective equipments that protect healthcare providers from infection and prevent the spread of infection from patient to patient.

Standard precautions are a combination of both universal precautions & body substance isolation precautions. All patient receiving care in hospital, irrespective of their diagnosis or presumed infection status, should be treated in a manner as to reduce the risk of transmission of microorganisms from HCW to patient.

- Perform hand hygiene
- Use personal protective equipment
- Follow respiratory hygiene
- Ensure appropriate patient placement
- Properly handle, clean & disinfect patient care equipments & instruments /devices
- Clean & disinfectant the environment appropriately
- Handle textiles & laundry carefully
- Follow safe injection practices
- Proper handling of needle & sharps

13. CARDINAL RULES OF STANDARD PRECAUTIONS:-

Consider all patients as potentially infectious. Assume all blood & body fluids & tissues covered by standard precautions as contaminated with blood borne pathogens. Assume all unsterile Needles & other sharps as similarly contaminated.

14. PROTOCOL FOR STANDARD PRECAUTION:-

- Hand washing for decontamination: With soap & water
- Hand disinfection: with chlorhexidine alcohol hand rubs or povidine iodine hand scrub.
- Use of personal protective equipment
- Handling needles & other sharps.

15. IS SOILED INFECTED LINEN SEGREGATED FROM OTHER LINEN AND HOW ?

Yes, soiled linen is soaked in 1% sodium hypochlorite for 1/2 hour and then sent to laundry in yellow bin/bag

16. WHAT IS DISINFECTION, DECONTAMINATION AND STERILIZATION ?

Disinfection:- It is as process by which vegetative i.e. growing forms of pathogenic organism are killed. This is done by using sodium, hypochlorite, alcohol, Lysol, bacciloidetc.

Decontamination:-It is a process by which vegetative micro-organisms are killed. Before cleaning such processing may be required to make solid instruments or material safe by using:-

- Detergent with enzyme for **scopes**.
- Chlorthidine with alcohol for preparation & disinfection of hands.
- Bacciloid for high risk area.

Sterilization :- It is the destruction of all micro-organism including bacterial pores. This is achieved by physical or chemical means e.g. saturated steam, ETO it is complete elimination or destruction of all forms of microbial life and is accomplished in health care facility by either physical or chemical process e.g. Autoclave/ETO.

17. WHAT IS BARRIER NURSING & REVERSE BARRIER NURSING?

Barrier Nursing :-

The nursing of patients with infectious diseases in isolation to prevent the spread of infection.As the name implies, the aim is to erect a barrier to the passage of infectious pathogenic organisms between the contagious patient and other patients and staff in the hospital, and thence to the outside world. The nurses wear gowns, masks, and gloves, and they observe strict rules that minimise the risk of passing on infectious agents.

Reverse Barrier Nursing:-

This includes precautions which need to be followed by HCWS, taking care of Immune compromised patients to prevent such patients from getting infections. in addition to standard precautions, it includes isolation in a positive pressure room.

DEFINE :-

A. ISOLATION

Separation of an ill person who has a communicable disease (e.g., measles, chickenpox, mumps, SARS) from those who are healthy. Isolation prevents transmission of infection to others and also allows the focused delivery of specialised health care to ill patients. The period of isolation varies from disease-to-disease. Isolation facilities can also be extended to patients for fulfilling their individual, unique needs.

B. INCIDENT REPORTING

It is defined as written or verbal reporting of any event in the process of patient care ,that is inconsistent with the deserved patient outcome or routine operations of the healthcare facility.

C. HAZARDOUS WASTE

Waste materials dangerous to living organisms. Such materials require special precautions for disposal. They include biologic waste that can transmit disease (for example, blood, tissues) radioactive materials, and toxic chemicals. Other examples are infectious waste such as used needles, used bandages and fluid soaked items.

D. HAZARDOUS MATERIALS

Substances dangerous to human and other living organisms. They include radioactive or chemical materials .

E. SURVEILLANCE

The continuous scrutiny of factors that determines the occurrence and distribution of diseases and other conditions of ill health. It implies watching over with great attention, authority and often with suspicion. It requires professional analysis and sophisticated interpretation of data leading to recommendations for control activities.